

Corporate Plant
Format

Title	Artwork Format		
Format No.	2002-0001-F0003-002		
Effective Date	30/07/2024	Page No.	1 of 1

SII

TETANUS TOXOID
VACCINE ADSORBED

Suspension for Injection

Therapeutic Code : J07AM01

DESCRIPTION
Tetanus toxoid vaccine adsorbed is prepared by detoxification of the sterile filtrate of broth cultures of Clostridium tetani with formalin and heat. The toxoid is purified by chemical methods and is adsorbed onto aluminium phosphate as adjuvant. Thiomersal is added as preservative. The vaccine has the appearance of a greyish-white suspension and does not contain any horse serum protein. Therefore it does not induce sensitization to sera of equine origin. The vaccine meets the requirements of W.H.O., EP and IP when tested by the methods outlined in W.H.O., TRS. 980 (2014), EP and IP.

POTENCY
Each single 0.5 ml human dose contains:
Tetanus Toxoid ≥ 5 Lf (≥ 40 IU)
Adsorbed on Aluminium Phosphate, Al+++ ≤ 1.25 mg.
Preservative : 0.005% Thiomersal

INDICATIONS
The vaccine is used for the prevention of tetanus in infants, children and adults, especially those liable to be exposed to tetanus infection and persons engaged in outdoor activities e.g. gardeners, farm workers and athletes.
Tetanus toxoid vaccine is also used in the prevention of neonatal tetanus by immunizing women of childbearing age, and also in the prevention of tetanus following injury. The vaccine can be safely and effectively given simultaneously with BCG, Measles, Polio vaccines (IPV and OPV), Hepatitis B, Yellow fever vaccine, Haemophilus influenzae type b, Varicella vaccine and Vitamin A supplementation.

APPLICATION AND DOSAGE
The full basic course of immunization against tetanus consists of two primary doses of 0.5 ml at least four weeks apart, followed by the third dose 6-12 months later. To maintain a high level of immunity further 0.5 ml booster doses are recommended at every feasible interval (for adults usually 5 to 10 years).

PROTECTION OF THE NEWBORN AGAINST TETANUS
For prevention of neonatal tetanus, tetanus toxoid is recommended for immunization of women of childbearing age, and especially pregnant women. Tetanus toxoid may be safely administered during pregnancy and should be given to the mother at first contact or as early as possible in pregnancy. A five dose schedule is recommended for previously unimmunized women of childbearing age : after the basic course of immunization with three doses, two additional booster doses should be given, at least one year after the previous dose or during the subsequent pregnancy.

VACCINATION OF INJURED PERSONS
For those subjects who have proof of either completing their course of primary immunizations containing tetanus toxoid or receiving a booster shot within the previous 5 years no additional dose of tetanus toxoid is recommended. If more than 5 years have elapsed, and infection with tetanus because of injury or other cause is suspected, 0.5 ml of the adsorbed tetanus toxoid should be given immediately. Where the immunization history is inadequate 1500 IU (3000 old AU) tetanus antiserum and 0.5 ml toxoid should be injected, with separate syringes, to different body sites. (If available, 250 units of tetanus immune globulin (human origin) can be substituted for the tetanus antiserum). A second 0.5 ml dose of toxoid is recommended after 2 weeks and a third dose after a further 1 month. (A note of caution : if horse-origin tetanus antiserum is used in prophylaxis, the patient should be tested for sensitivity to horse serum protein prior to its administration. It is desirable to have 1 ml of Adrenaline solution (1 : 1000) immediately available and the normal precautions followed when injecting antitoxins).

METHOD OF INOCULATION
Tetanus toxoid should be injected intramuscularly into the deltoid muscle in women and older children. If there are indications for the use of tetanus toxoid in younger children, the preferred site for intramuscular injection is the anterolateral aspect of the upper thigh since it provides the largest muscular mass. Only sterile needles and syringes should be used for each injection. The vaccine should be well shaken before use.
Once opened, multi-dose vials should be kept between +2 °C and +8 °C. Multi-dose vials of Tetanus toxoid vaccine adsorbed from which one or more doses of vaccine have been removed during an immunisation session may be used in subsequent immunisation sessions for upto a maximum of 28 days, provided that all of the following conditions are met (as described in the W.H.O. policy statement: Handling of multi dose vaccine vials after opening, W.H.O./IVB/14.07):

- The vaccine is currently prequalified by W.H.O.;
- The vaccine is approved for use for up to 28 days after opening the vial, as determined by W.H.O.;
- The expiry date of the vaccine has not passed;
- The vaccine vial has been, and will continue to be, stored at W.H.O.- or manufacturer recommended temperatures; furthermore, the vaccine vial monitor, if one is attached, is visible on the vaccine label and is not past its discard point, and the vaccine has not been damaged by freezing.

The vaccine should be visually inspected for any foreign particulate matter and / or variation of physical aspect prior to administration. In event of either being observed, discard the vaccine.

REACTIONS
Reactions are generally mild and confined to the site of injection. Some inflammation may occur together with transient fever, malaise and irritability. Occasionally a nodule may develop at the site of injection but this is rare. An increased severity of reactions to vaccination may be observed in subjects who have had many booster immunizations.

CONTRAINDICATIONS AND WARNINGS
The vaccine should not be given to persons who showed a severe reaction to a previous dose of tetanus toxoid Immunisation should be deferred during the course of any febrile illness or acute infection. A minor febrile illness such as a mild upper respiratory infection should not preclude immunisation. Risk of sensitization in relation to thiomersal and other preservatives.

PRECAUTIONS
ADRENALINE INJECTION (1:1000) MUST BE IMMEDIATELY AVAILABLE SHOULD AN ACUTE ANAPHYLACTIC REACTION OCCUR DUE TO ANY COMPONENT OF THE VACCINE. For treatment of severe anaphylaxis the initial dose of adrenaline is 0.1 - 0.5 mg (0.1-0.5 ml of 1:1000 injection) given s/c or i/m. Single dose should not exceed 1 mg (1 ml). For infants and children the recommended dose of adrenaline is 0.01 mg/kg (0.01 ml/kg of 1:1000 injection). Single paediatric dose should not exceed 0.5 mg (0.5 ml). The mainstay in the treatment of severe anaphylaxis is the prompt use of adrenaline, which can be lifesaving. It should be used at the first suspicion of anaphylaxis.
As with the use of all vaccines the vaccinees should remain under observation for not less than 30 minutes for possibility of occurrence of immediate or early allergic reactions. Hydrocortisone and antihistaminics should also be available in addition to supportive measures such as oxygen inhalation. There is an increased incidence of local and systemic reactions to booster doses of tetanus toxoid when given to previously immunized persons.
Special care should be taken to ensure that the injection does not enter a blood vessel.
IT IS EXTREMELY IMPORTANT WHEN THE PARENT, GUARDIAN, OR ADULT PATIENT RETURNS FOR THE NEXT DOSE IN THE SERIES, THE PARENT, GUARDIAN, OR ADULT PATIENT SHOULD BE QUESTIONED CONCERNING OCCURRENCE OF ANY SYMPTOMS AND/OR SIGNS OF AN ADVERSE REACTION AFTER THE PREVIOUS DOSE.

DRUG INTERACTIONS
If passive immunisation for tetanus is needed, TIG (Human) is the product of choice. It provides longer protection than antitoxin of animal origin and causes few adverse reactions.
As with other Intramuscular injections, use with caution in-patients on anticoagulant therapy. Immunosuppressive therapies may reduce the immune response to vaccines.

HIV INFECTION
TT vaccine may be used in children with known or suspected HIV infection. Although the data are limited and further studies are being encouraged, there is no evidence to date of any increased rate of adverse reactions using this vaccine in symptomatic or asymptomatic HIV infected children.

STORAGE OF THE VACCINE.
The vaccine should be stored in a dry, dark place at a temperature between 2-8 °C. Transportation should also be at 2-8 °C. DO NOT FREEZE.

SHELF LIFE
Thirty six months from the date of manufacture.

PRESENTATION
1 dose ampoule of 0.5 ml
10 dose vial of 5 ml
20 dose vial of 10 ml

THE VACCINE VIAL MONITOR (Optional)

Vaccine Vial Monitors (VVM)

USE

 Square is lighter than outer circle

The colour of the inner square of the VVM starts with a shade that is lighter than the outer circle and continues to darken with time and/or exposure to heat.

DO NOT USE

 Square matches outer circle

DISCARD POINT

 Square is darker than outer circle

Once a vaccine has reached or exceeded the discard point, the colour of the inner square will be the same colour or darker than the outer circle.

Inform your supervisor

Cumulative heat exposure over time



Manufactured by:

SERUM INSTITUTE OF INDIA PVT. LTD.

S. Nos. 203-205, 212, 223, 268-270 (H) & 104-110,
113-114, 168-170 (M), Hadapsar, Pune 411 028, INDIA

Protection from birth onwards

20013105/01

Reason for issue: SII logo and address revised		Specification: To be printed on bible paper 40 gsm.					
Customer: Malaysia							
Product: TETANUS TOXOID VACCINE ADSORBED Name							
Item Code number: 20013105/01		Specification No.:	Artwork made to: 100%				
Supercedes Item Code: 20013105/0		Dimensions: 123 x 250 mm					
Prepared By	Reviewed By	Approved By		Authorized By			
PACKAGING DEVELOPMENT	QUALITY CONTROL	REGULATORY AFFAIRS	CLINICAL RESEARCH & PHARMACOVIGILANCE	QUALITY ASSURANCE			

File Name: E:\Artworks NL\Hadapsar\Insert\Export\Malaysia\TT vaccine insert - NL.cdr

Date: 06.02.2026

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